12.1.4 Prevention and control of vitamin A deficiency

1. Nutrition education
2. Dietary modification: The most rational and sustainable long term solution to control of vitamin A deficiency is to ensure that the community includes foods rich in vitamin A or its precursor regularly in their daily diets.
3. Periodic supplementation of vitamin A: Currently the massive vitamin A supplementation programme aims at providing the first dose of 1,00,000 IU at 9 months (at the time of measles immunization) to be followed by bi annual administration of 2,00,000 IU for children between the ages of 18 months and 59 months. The coverage under massive dose vitamin A administration has improved substantially after the initiation of biannual administration.
4. Fortification of commonly and widely consumed foods with vitamin A: Fortification or enrichment of widely consumed foods with vitamin A is another strategy to prevent and control vitamin A deficiency. Foods which are consumed daily by all sections of the community with little variation in the intake are generally utilized for the fortification. Fortified foods are integrated into the conventional food system as value added products to reach a large segment of population.

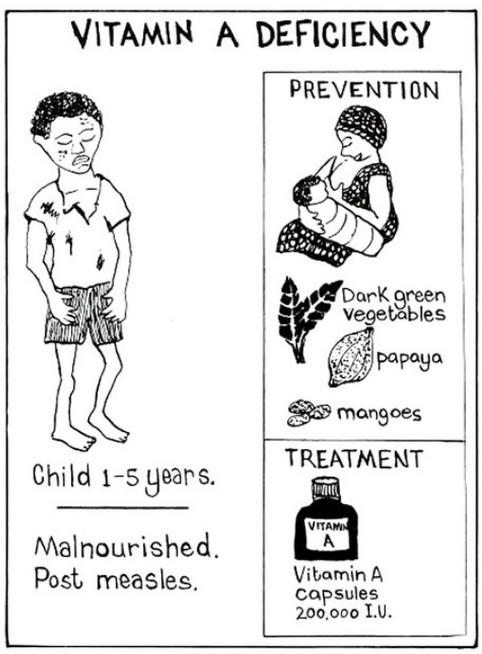


Fig 12.3: Vitamin a dose

12.2 Food Fortification Programme

Fortification of food items such as wheat flour, bread, milk, sugar, drinking water and common salt are in practice in different parts of the world.

Iodised salt: Common salt has been selected as a suitable vehicle for fortification of iodine to control IDD(Iodine Deficiency Disorder). The technology involved in fortification of salt with iodine involves either dry mixing or spray mixing of salt with iodine source mainly with potassium iodate. It is an economical, convenient



Fig 12.4: Iodised salt

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